INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Pembrolizumab every 6 weeks for head and neck cancer

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
42 days
See the active treatment phase

Treatment stages
1
Explained in the treatment reference

Regimen source
NCCP 00558
Version 13

Every 42 days; duration individually reviewed

Treatment stages

01 **Pembrolizumab monotherapy**
Every 6 weeks

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

		PAGE
01	Understanding your treatment	2
02	Your schedule, tests and appointments	5
03	Symptoms and supportive care	11
04	Sources and further information	18

My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Contact oncology immediately for new breathlessness, chest pain, diarrhoea, severe tummy pain, yellow skin or eyes, much less urine, severe headache, vision changes, marked weakness, confusion, unusual thirst or a spreading or blistering rash. Severe breathing difficulty, collapse or seizure is an emergency.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

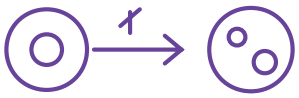
Your treatment medicines and modalities

Pembrolizumab aims to control head and neck cancer that has returned or spread, relieve symptoms and help some people live longer. Benefit and side effects are reviewed for you.

IMMUNOTHERAPY

Pembrolizumab

Keytruda



Blocks an immune brake

Pembrolizumab is an antibody that blocks PD-1, a control switch on immune T cells. Cancer cells can use this switch to reduce immune attack. Blocking it can help the immune system recognise and attack cancer. The same increased immune activity can inflame healthy organs, which is why new symptoms need attention even after treatment ends.

Immune cell → PD-1 blocked → Cancer attacked

What to watch for

Report new breathing, bowel, skin, hormone, heart or nerve symptoms promptly; these may be immune inflammation.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Your diagnosis and treatment aim

Keep your own diagnosis and treatment aim at the front of the plan.

The setting for this guide

First-line treatment of metastatic or unresectable recurrent head and neck squamous cell carcinoma with a tumour PD-L1 combined positive score (CPS) of at least 1. This indication excludes nasopharyngeal cancer and cancer progressing within six months after curatively intended systemic treatment for locoregionally advanced disease.

Choosing between treatments

Your team weighs the cancer, previous treatment, health, symptoms and preferences when discussing options. Other systemic treatment, local treatment where suitable, trials or symptom-focused care may be discussed.

How your treatment works

Cancer treatment can also affect healthy tissue.

Evidence and further information

Use evidence to support a personal treatment discussion.

What studies can tell us

KEYNOTE-048 supports pembrolizumab alone in the PD-L1-positive first-line recurrent or metastatic population. It compared treatment groups, not an individual prognosis. The trial used 200 mg every three weeks; the six-week schedule is not a separate randomised head and neck survival comparison.

Your individual decision

Group results cannot predict what will happen for you. Ask which benefits matter for your situation and how the team will decide whether treatment remains worthwhile.

Resources

The source list gives the official regimen and public patient information. Ask your team for a printed copy or help accessing a resource if needed.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

Your overall treatment plan

Pembrolizumab every 6 weeks for head and neck cancer

42-day view



This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

EVERY 6 WEEKS

Pembrolizumab monotherapy

Day 1 every 42 days | **Duration individually reviewed**

Pembrolizumab 400 mg is given through a vein over about 30 minutes on Day 1 every 6 weeks. The whole visit includes safety checks, preparation and observation. Treatment continues until progression or unacceptable toxicity. After 24 months of sustained response without progression, the team may consider stopping; stopping at 24 months is not automatic.

Treatment continues until progression or unacceptable toxicity. After 24 months of sustained response without progression, the team may consider stopping; stopping at 24 months is not automatic.

BASELINE → TREATMENT → FOLLOW-UP

Required safety checks and your individual cancer-assessment plan

These stages are not equal time intervals. Your team provides dates and reviews results. There is no universal blood tumour-marker panel or scan interval in this guide.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00558 v13: official regimen

Tests in your plan

Full blood count

WHEN

Before starting and before each treatment cycle

Checks blood cells and helps assess infection, anaemia and bleeding risk.

Kidney and liver tests

WHEN

Before starting and before each cycle

Checks for organ injury, including possible immune inflammation.

Blood glucose

WHEN

Baseline and before each cycle

Checks blood sugar and possible treatment-related diabetes.

Thyroid function

WHEN

Baseline; every 3–6 weeks during treatment

Checks for an underactive or overactive thyroid.

Cortisol

If part of your plan

WHEN

When clinically indicated

Assesses possible adrenal or pituitary problems when symptoms or findings suggest them.

Tumour PD-L1 combined positive score

WHEN

Before choosing treatment

Checks whether the cancer meets this monotherapy indication; CPS is different from the lung tumour proportion score.

Scans and head and neck examination

If part of your plan

WHEN

As prescribed in the individual disease-assessment plan

Assesses the cancer and the response to treatment separately from safety bloods.

THE WHOLE TREATMENT JOURNEY

Pembrolizumab every 6 weeks for head and neck cancer: treatment and checks

Pembrolizumab 400 mg is given through a vein over about 30 minutes on Day 1 every 6 weeks. The whole visit includes safety checks, preparation and observation. Treatment continues until progression or unacceptable toxicity. After 24 months of sustained response without progression, the team may consider stopping; stopping at 24 months is not automatic.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	During treatment	Between visits	Response review	After treatment
Bloods and organ safety NCCP	Full blood count, kidney and liver tests, glucose and thyroid tests; review autoimmune disease, transplants, previous immune side effects and all medicines. Confirm the tumour PD-L1 combined positive score; this is a tumour test, not a routine blood test.	Full blood count, kidney and liver tests and glucose before every 6-week cycle. Thyroid tests every 3–6 weeks; cortisol when clinically indicated.	New symptoms may require extra tests before the next planned review.	Safety bloods do not by themselves show whether treatment controls the cancer.	Keep reporting new symptoms after treatment ends. Steroid tapers, hormone replacement or specialist review may continue longer. Scans and examinations follow your individual response and follow-up plan.
Thyroid and other hormones NCCP	Baseline thyroid function and glucose.	Thyroid tests every 3–6 weeks, including between six-week visits when due.	Report marked thirst, unusual fatigue, dizziness, headache or weakness.	Cortisol or other hormone tests when clinically indicated.	Continue prescribed hormone replacement and monitoring when needed.
Head and neck function NCCP	Record weight, intake, swallowing, speech and existing mouth or dental problems.	Review pain, intake, weight and support needs.	Report choking, inability to drink, worsening swallowing or new neck symptoms.	Agree examinations and scans with the oncology team.	Continue rehabilitation and symptom support as needed.
Medicines and practical plan NCCP	Confirm the exact prescription, treatment aim and actual urgent contacts.	Bring your medicine list, current support plan and treatment record.	Record symptoms and medicines taken; ask before adding a new product.	Confirm any pause, dose change or stop in writing.	Record the final dose and the teams responsible for follow-up.

What each check is for

1 **Pembrolizumab monotherapy**

Every 6 weeks

Pembrolizumab aims to control head and neck cancer that has returned or spread, relieve symptoms and help some people live longer. Benefit and side effects are reviewed for you.

Ask / act: Confirm the current phase, prescription, tests and support plan.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Your practical treatment plan

STEP 3

Tests and review

Full blood count, kidney and liver tests and glucose before every 6-week cycle. Thyroid tests every 3–6 weeks; cortisol when clinically indicated.

STEP 4

Home record

Carry your treatment record, actual urgent contacts and medicine list. Tell the team about line redness, swelling, leakage, pain or fever.

STEP 5

Follow-up

Keep reporting new symptoms after treatment ends. Steroid tapers, hormone replacement or specialist review may continue longer. Scans and examinations follow your individual response and follow-up plan.

Your treatment schedule

Keep one calendar for treatment, tests and other appointments.

A pause or change

Your team may pause or stop treatment to assess side effects. Record the reason, required tests and who can authorise restarting. A new cycle does not erase earlier severe toxicity.

Medicines, precautions and visit preparation

Review medicines and practical needs before treatment.

Medicines and fertility

Bring prescribed, over-the-counter and herbal medicines and supplements for review. Discuss fertility preservation and breastfeeding before treatment. Use effective contraception during treatment and for at least four months after the final dose. Tell the team promptly about a possible pregnancy. Do not breastfeed without specific advice from the team.

Access and observation

Your team chooses suitable venous access for an infusion. A port, if used, has its own insertion, care and removal plan; it is not removed after each visit. No take-home chemotherapy pump is part of this regimen.

Work, transport and communication

Discuss transport, work, caring responsibilities, interpreter needs and a support person. Ask how results and changes will be communicated and who books your next tests.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Day 1 → Day 42: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Pembrolizumab treatment	 **Day 1** Pembrolizumab 400 mg is given through a vein over about 30 minutes on Day 1 every 6 weeks. The whole visit includes safety checks, preparation and observation.
Symptoms between visits	 **Days 1–42** Immune side effects can occur on any day, including after treatment stops.
Prepare next review	 **Day 42** Arrange required tests before the next dose; timing and results must be confirmed by your team.

Treatment sequence: Treatment continues until progression or unacceptable toxicity. After 24 months of sustained response without progression, the team may consider stopping; stopping at 24 months is not automatic.

After treatment: Keep reporting new symptoms after treatment ends. Steroid tapers, hormone replacement or specialist review may continue longer. Scans and examinations follow your individual response and follow-up plan.

> **Why these bands stay broad:** This is an illustrative cycle, not a prediction of safe days or a substitute for the appointment calendar. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment.
> NCCP 00558 v13: official regimen ↗

> **There are no guaranteed “safe days”.** This information supports discussion; your oncology team confirms the personal prescription and urgent-care plan.

Between-treatment notes

Day 1: Treatment visit

Pembrolizumab 400 mg is given through a vein over about 30 minutes on Day 1 every 6 weeks. The whole visit includes safety checks, preparation and observation. Report symptoms and previous reactions before treatment.

Day 2–42: Between visits

Immune side effects may start between visits or after the final dose. Report changes promptly. Follow the support plan and contact oncology early if symptoms develop.

Support around the medicines

1 **Immune alert card and treatment holds**

Carry your immune alert card during and after pembrolizumab. Tell any clinician that you have received immunotherapy. Do not restart a held treatment or change prescribed steroids or hormone replacement yourself.

2 **Eating and weight**

Tell the team early if swallowing, appetite, pain or taste changes reduce intake. A dietitian can agree practical food, drink or feeding support with you. Do not wait for substantial weight loss.

3 **Mouth and dental care**

Agree a mouth-care plan and tell your dentist about previous or current cancer treatment. Arrange oncology advice before dental procedures, particularly after head and neck radiation.

4 **Swallowing and communication**

Ask for speech and language therapy if swallowing or speech is difficult. Use the food textures, exercises and communication support advised for you; a general guide cannot establish safe swallowing.

5 **Medicines and fertility**

Tests and review

Blood safety tests, tumour tests and scans answer different questions.

Before the first dose

Full blood count, kidney and liver tests, glucose and thyroid tests; review autoimmune disease, transplants, previous immune side effects and all medicines. Confirm the tumour PD-L1 combined positive score; this is a tumour test, not a routine blood test.

Response assessment

Your team agrees examinations and scans to assess cancer response. No universal scan interval or blood tumour-marker panel is assumed here.

Recognising side effects

Use the anatomical map with these explanations; location does not tell you how urgent a symptom is.

Hormones / nervous system

Severe headache, confusion, weakness, dizziness or marked thirst need prompt advice.

Lungs

New cough or breathlessness can signal inflammation or another urgent cause.

Heart

Chest pain, racing heartbeat or fainting needs urgent assessment.

Liver

Yellow eyes/skin or dark urine may signal inflammation; tests also check the liver.

Kidneys

Report much less urine, new swelling or a change in kidney tests.

Bowel

New or persistent diarrhoea, blood in stools or severe tummy pain needs urgent advice.

Skin

Report a spreading rash, painful skin, blisters or peeling promptly.

Muscles / nerves

Severe weakness, drooping eyelids, swallowing or breathing difficulty can be urgent immune effects.

Eyes / vision

New blurred vision, painful eyes or visual change needs prompt advice.

When and how to get help

Use the oncology service's written urgent-contact plan.



During an infusion or injection

Tell staff immediately about itching, a rash, dizziness, chest tightness, breathing difficulty, pain or swelling around the treatment site.

Prepare the handover

Tell the clinician the medicine name, date of the last dose, new symptoms and any prescribed steroids or hormone replacement. Carry the wallet card; do not delay urgent care while looking for a document.

Eating, mouth care and everyday support

Ask for help before a practical problem prevents treatment.



Eating and weight

Swallowing and communication

Ask for individual speech and language therapy advice for swallowing or communication difficulties. Report choking, coughing with drinks or inability to swallow safely.

Energy and activity

Plan manageable activity and rest around symptoms. Tell the team about severe or unexpected fatigue, falls or difficulties managing at home.

Your record and contact card

Keep the information that another clinician would need.



Cycle or review record

Record the cycle received, date, route, dose if supplied, important symptoms, any hold and the next agreed review. Stamp boxes record treatment received; they do not authorise the next dose.

Contacts and documents

Have the team replace example telephone numbers with your actual daytime and out-of-hours contacts. Carry the front/back wallet card and immune alert card when applicable. The printed handbook contains the key information without needing to scan a QR code.



Questions for review

Bring your main question, changes in swallowing or weight, symptoms, medicine changes and any concerns about the treatment aim. Ask for the plan in words you can explain back.

Questions worth asking

What happens if treatment is held?

The team records the reason, required tests and recovery and who can authorise restarting. Do not count a held dose as received.

Try explaining the plan back: Explain your treatment schedule in your own words.,Name the symptoms that mean you should call.,Point to the actual urgent contact number and the next required tests.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Pembrolizumab monotherapy

Active medicines: Pembrolizumab

Emetogenic risk: Minimal

Incomplete clinician/pharmacy review draft. Individualise and authorise before use.

Additional phase-specific support

- Routine preventive anti-sickness or steroid medicines are not usually needed for this monotherapy. Steroids or hormone replacement may be prescribed for a specific complication; they are not a generic prophylactic prescription.
- Carry your immune alert card during and after pembrolizumab. Tell any clinician that you have received immunotherapy. Do not restart a held treatment or change prescribed steroids or hormone replacement yourself.
- Tell the team early if swallowing, appetite, pain or taste changes reduce intake. A dietitian can agree practical food, drink or feeding support with you. Do not wait for substantial weight loss.
- Agree a mouth-care plan and tell your dentist about previous or current cancer treatment. Arrange oncology advice before dental procedures, particularly after head and neck radiation.
- Ask for speech and language therapy if swallowing or speech is difficult. Use the food textures, exercises and communication support advised for you; a general guide cannot establish safe swallowing.
- Bring prescribed, over-the-counter and herbal medicines and supplements for review. Discuss fertility preservation and breastfeeding before treatment. Use effective contraception during treatment and for at least four months after the final dose. Tell the team promptly about a possible pregnancy. Do not breastfeed without specific advice from the team.
- Keep reporting new symptoms after treatment ends. Steroid tapers, hormone replacement or specialist review may continue longer. Scans and examinations follow your individual response and follow-up plan.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Hormones / nervous system

Pembrolizumab

Severe headache, confusion, weakness, dizziness or marked thirst need prompt advice.

3 Heart

Pembrolizumab

Chest pain, racing heartbeat or fainting needs urgent assessment.

5 Kidneys

Pembrolizumab

Report much less urine, new swelling or a change in kidney tests.

7 Skin

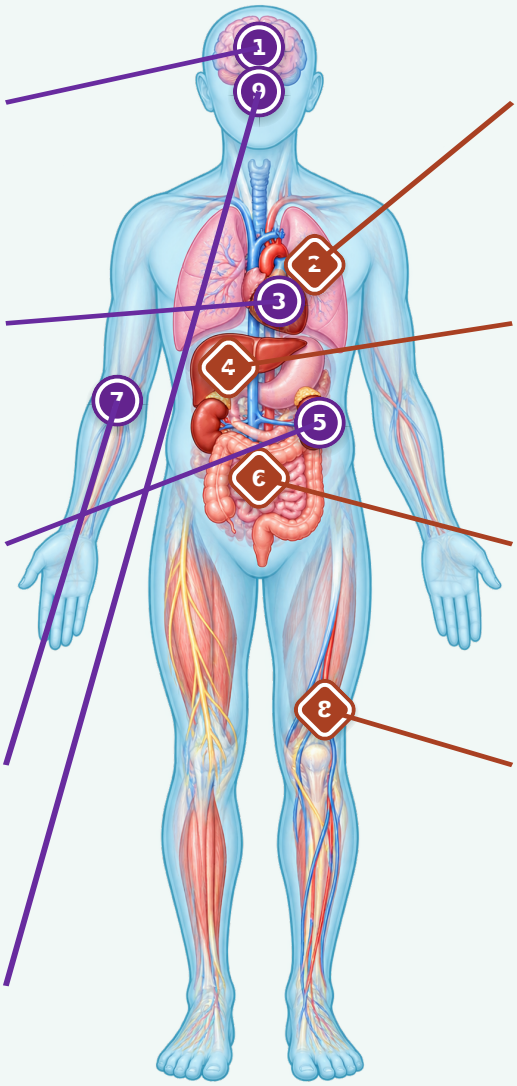
Pembrolizumab

Report a spreading rash, painful skin, blisters or peeling promptly.

9 Eyes / vision

Pembrolizumab

New blurred vision, painful eyes or visual change needs prompt advice.



2 Lungs

Pembrolizumab

New cough or breathlessness can signal inflammation or another urgent cause.

4 Liver

Pembrolizumab

Yellow eyes/skin or dark urine may signal inflammation; tests also check the liver.

6 Bowel

Pembrolizumab

New or persistent diarrhoea, blood in stools or severe tummy pain needs urgent advice.

8 Muscles / nerves

Pembrolizumab

Severe weakness, drooping eyelids, swallowing or breathing difficulty can be urgent immune effects.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Contact oncology immediately for new breathlessness, chest pain, diarrhoea, severe tummy pain, yellow skin or eyes, much less urine, severe headache, vision changes, marked weakness, confusion, unusual thirst or a spreading or blistering rash. Severe breathing difficulty, collapse or seizure is an emergency.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **NCCP 00558 v13 · pNCCP treatment, tests and supportive-care sections**
   https://healthservice.hse.ie/documents/6389/Pembrolizumab_400mg_monotherapy_558_.pdf
2. **NCI: head and neck cancers**
   https://www.cancer.gov/types/head-and-neck/head-neck-fact-sheet
3. **NCI: oral complications of cancer therapies, patient version**
   https://www.cancer.gov/about-cancer/treatment/side-effects/mouth-throat/oral-complications-pdq
4. **NCI: nutrition during cancer treatment**
   https://www.cancer.gov/about-cancer/treatment/side-effects/nutrition
5. **EMA Keytruda intravenous (IV) product information and patient leaflet**
   https://www.ema.europa.eu/en/documents/product-information/keytruda-epar-product-information_en.pdf
6. **KEYNOTE-048: original trial report**
   https://pubmed.ncbi.nlm.nih.gov/31679945/